Name: Karter M Lundy
DOB: 3/1/2020
Date: 10/9/2026
Ethnicity: White
Pronouns: He/Him

Start time: 10:00 AM
End time: 11:14 AM
CPT Code: 90791 GT
Family/Others Present: Yes:father

LOCATION OF SERVICE
This encounter was provided from my home office via live, two-way video conferencing. The patient's name was verified, and it was confirmed that the patient was located in their home within Pennsylvania. The patient did not have assistance. Verbal consent was obtained from the patient and/or their caregiver to participate in telehealth services. The patient and/or their caregiver was/were informed of the following:
• The equipment/technology used to provide the service
• All persons present at each end of the transmission and their role(s) in the service
• That they may refuse services via telehealth and that such refusal will not be used as a basis to limit their access to other available services
• Alternatives to telehealth services and the associated possible delays in service, need to travel, or risks associated with not having the services provided via telehealth

CONSENT FOR EVALUATION
Jeffrey Unterberger, PsyD obtained informed consent from Karter M Lundy and/or their legal guardian(s) to conduct the evaluation. Jeffrey Unterberger, PsyD identified themselves and their role and reviewed:

Expected procedures;
Potential risks and benefits of the service;
Patient financial obligations;
Patient right's and responsibilities; and
Patient privacy, steps that are taken to keep protected health information private, limits of confidentiality, and who has access to the patient's record (i.e., the patient and their proxy, if there is one, agency staff who have medical or business need to access the record, and individuals to whom the patient has released information).

Karter M Lundy and/or their legal guardian(s) were given the opportunity to ask questions, which were answered to their satisfaction. Informed consent to proceed with the evaluation was given by the patient's father. Verbal consent was also given my patient's mother via phone prior to the appointment.

PRESENTING PROBLEM/REASON FOR REFERRAL:

Karter is a 6-year-old male who was referred for a psychological evaluation by Katie Rich, CRNP, due to concern for the possible presence of ADHD. The purpose of the current assessment was to identify Karter's strengths and weaknesses, provide diagnostic clarity regarding Karter's symptoms, and to offer recommendations for treatment and intervention.

MEDICAL HISTORY:
Karter's mother experienced no complications during pregnancy with Karter. However, there was exposure to Zoloft, tobacco, and THC during pregnancy per medical record review. There was delivery at term, and Karter's birth weight was 7 lbs., 6 oz. Karter was a healthy newborn and received standard care.

Karter has an estimated body mass index of 17.52 kg/m². Medical history is remarkable for asthma. Further, Karter's father notes him needing stitches on his face and breaking his collar bone on separate occasions of falling when playing. Hearing and vision are typical. There is no history of concussion/serious head injury or seizure activity. Karter is prescribed an albuterol inhaler. He also takes Zyrtec. Karter does not have any known food or medication allergies. Regarding sleep, sleeps well. Appetite is typical, though he is a very selective / picky eater. There is no history of feeding difficulty or disordered eating.

FAMILY HISTORY:
Karter primarily lives with lives with his father. At father's home, Karter also lives with father's girlfriend and her two sons (ages 7 and 11), as well as a newborn brother. Karter stays with mother during the weekends where his younger sister also lives. . Parents are separated (never married). He has two older half-brothers (they live with their biological-father). There is a formal custody arrangement. Family psychiatric history is remarkable for ADHD, anxiety, and depression in father, depression and anxiety in mother.

DEVELOPMENTAL HISTORY:
Developmental milestones (speech, motor, adaptive): met milestones within normal limits per dad's recollection. Chart review indicates speech delay. Chart review indicates that around 2 "only says a handful of words ~ 10. Not stringing words together."
Currently, dad reports that Karter tends to speak very quietly, tends to stutter, speech is choppy. Seems to have plateaued more with his speech around age 5. Family is starting to get him into speech
Early intervention services Denied- chart indicates that family was considering EI evaluation at one point but ultimately did not pursue it due to minimal concerns.
Developmental Regression: Denied
Social Responsiveness (response to name): mildly endorsed - when hyper-focused on a task
Interest in/engagement with same-age peers: seems socially motivated, gets upset when kids to not play the way he wants them to. Can be controlling / directive in his play.
Play skills/use of toys: Denied concerns

PSYCHIATRIC HISTORY:

Psychiatric diagnoses: Denied
Previous psychological evaluations: Denied; PCP did vanderbilt; suggestive of ADHD, but family was interested in a fully evaluation for more certainty
Current/past treatment providers: Denied
History of psychiatric inpatient hospitalizations: Denied
History of past suicide attempts: Denied

ACADEMIC HISTORY:
Current grade:1
School/District: Fox Chapel
History of receiving academic accommodations: Denied
Grades: dad reports that he no longer seems interested in learning, though he appeared more eager to learn during Prek years. His academic skills seem on track, but dad reports that he tends to try and avoid work by saying he is unsure what to do
Skipping/repeating grade: n/a
Behavioral/emotional problems/teacher concerns: struggles to stay on task, being disruptive,
Suspensions/expulsions: n/a

SOCIAL HISTORY:
Friendships: Tends to get along well with GF's son (who is 7). Talks to dad about friend at school, seems to want to have play dates.
Conflict/bullying: some concerns for bullying in K (was being bullied by a 5th grader on the bus), but no longer a concern (this student has moved to the middle school)
Romantic relationship: n/a
Interests/hobbies/extracurricular activities: Paw Patrol, Sonic, Mario, Mine Craft

RELIGIOUS/SPIRITUAL/CULTURAL CONSIDERATIONS:
Denied

VOCATIONAL HISTORY:
None

MILITARY HISTORY:
None

LEGAL HISTORY:
None

SUBSTANCE USE HISTORY:
None

TRAUMA:

Grandmother passed away Nov 2024, Karter was with his father when grandmother was found dead. Will express missing her on occasion. Reports some stressors related to navigating custody.

EMOTIONAL/BEHAVIORAL HISTORY:
Autism Spectrum Disorder:

Social Communication/Interaction
Reciprocal conversation skills: Restricted to his own interests. May monopolize conversations "info dumps." Has trouble sustaining other conversation topics. Occasionally will ask of others' experiences, though infrequent.
Sharing of emotions/affect: described as inconsistent. Can "shut down" when discussing feelings.
Initiation/response to social interactions: Comes on strong, gives brief responses when interested
Eye contact, gestures, facial expressions: overall typical
Social Awareness (detecting sarcasm, social cues, non-verbals): can struggle to "read the room" such as when his behaviors are upsetting or bothering other children.
Empathy/perspective taking: can be empathetic, will offer comfort when siblings are upset

Restricted Interests/Repetitive or Stereotyped Behaviors
Motor movements: Endorsed, though dad describes more so hyperactivity (e.g., "always moving around").
Use of objects: Denied
Atypical speech/language: low volume, choppy cadence. Some repetitive speech reported by dad, can be hard to follow along with what he says (e.g., describes events out of correct sequence)
Insistence on sameness/routine: tends to get upset, though can adjust within a few minutes.
Rigidity/inflexibility: Tends to be very particular about his routines and following steps in a certain sequence; particular about foods (e.g., had been opposed to family trying to eat healthier) Can be rigid about rules, especially when playing games
Restricted/fixated interests: Denied
Sensory aversions: loud noises, certain textures on his hands (e.g., smiles)
Sensory seeking behaviors/hypo-reactivity: proprioceptive sensory input

Depression: Denied. More so struggles with regulation. Tends to be very bright and cheerful until something does not go his way - "shuts down and ignores"

Mania: Denied.

Anxiety Symptoms - has seemed to be a bit more cautious since breaking his collar bone, though denied anxiety overall. Gets "quiet" when in trouble.
Generalized Anxiety Symptoms: Absent

Social Anxiety Symptoms: Absent
Panic Symptoms: Absent
Agoraphobia Symptoms: Absent
Specific Phobia Symptoms: Absent
Separation Anxiety Symptoms: Denied

Obsessions/Compulsions:
OCD Symptoms: Absent
Hoarding Symptoms: Absent
Body Dysmorphia Symptoms: Absent
Hair-Pulling/Skin-Picking Symptoms: Absent

Attention & Concentration:
ADHD Diagnostic Criteria:
Inattention:

1. *Often make careless errors?* Endorsed (e.g., rushes through things)
2. *Trouble focusing for long periods of time? Endorsed*
3. *Trouble listening during conversations? Endorsed*
4. *Easily sidetracked from tasks and don't finish them? Endorsed (needs a lot of reminders to stay on track)*
5. *Trouble organizing tasks? Endorsed*
6. *Avoid tasks that require a lot of mental effort? Endorsed*
7. *Often lose important things? Endorsed*
8. *Easily distracted by noise/thoughts/stimuli? Endorsed*
9. *Often forgetful in daily activities? Denied*

Hyperactivity/Impulsivity:

1. *Often fidget? Endorsed*
2. *Getting out of Seat? Endorsed*
3. *Running/climbing? Endorsed*
4. *Trouble staying quiet? Endorsed*
5. *Often "on the go"? Endorsed*
6. *Overly talkative? Mildly endorsed "sometimes"*
7. *Often cut off others in conversation? Endorsed*
8. *Trouble waiting? Endorsed*
9. *Often intrude or interrupt others? Endorsed*

Disruptive Behaviors/Oppositionality/Conduct:

- Looks like: crying, yelling
- Onset: since he was a toddler
- Frequency: a few times a day
- Average duration: usually just a few minutes
- Antecedents/Triggers: does not get his way, someone not following the rules

- Settings: home & school
- Rates of compliance: struggles with compliance, refuses or ignores his father. May require several prompts depending on the task.

Perceptual Disturbances: Denied.